Mp10 Dashboard — User's Guide

The cards on the Billing and Patients main windows, and what to do about what they show

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1 About this guide

The Billing module (PrBill10 or MpBill10) and the Patients module (Patients10) open on a **dashboard**: a set of cards drawn on the main window that tell you, at a glance, what needs your attention today. Claims a check has flagged, encounters that were never billed, rejections waiting to be worked, a backup that did not run — each of these used to be something you had to go looking for. The dashboard puts them in front of you as soon as you sign in.

This guide is for the people who use those modules every day: billers, front desk and admissions staff, and the record room. It explains what each card means, which lines you can click, and what to do about what they show.

Two parts deserve particular attention, and each has its own chapter:

- **Claims with error tags** — how to open a flagged claim and find out exactly why it was flagged. The chapter after it, **The claim checks**, lists every check and the tag it places.
- **Unbilled encounters** — what they are, why they matter, and how to clear the ones that will never be billed because the patient did not show up.

2 Using the dashboard

2.1 When it appears

The dashboard shows whenever **no grid and no option window is open** on the main window. Open the Patients grid, the Claims grid, or any other window, and the cards step aside; close it, and they come back.

You do not have to close your work to see the dashboard. Press the **Dashboard** button on the toolbar (or choose **Dashboard** from the menu). Every open grid is hidden — not closed — and the cards appear. Each grid comes back exactly as you left it when you press its own toolbar button again.

2.2 Which cards you see

Each card reads certain tables. If your user account is not allowed to read one of them, that card (or that line of a card) is simply left out. Two people at the same site can therefore see different dashboards; that is expected and is not a fault. If you believe you should be seeing a card you do not, ask your administrator to review your permissions.

Some lines also depend on what your site uses. The HL7 line appears only at a site that runs an HL7 interface, the modality worklist line only where the worklist is installed, and so on.

2.3 How fresh the numbers are

The numbers are gathered in the background, so the dashboard never slows down the window you are working in. They refresh:

- about every **five minutes** while the dashboard is showing, and
- every time you **come back** to the dashboard after working in a grid, so what you just changed is reflected.

When you first sign in, a card may say **Loading...** for a few seconds.

If a card's bottom line turns orange and reads **"Numbers as of hh:mm:ss · the refresh has stopped"**, the numbers on screen are old. Close and reopen the module. If it keeps happening, tell your administrator.

A card that shows a **red message** instead of numbers could not read its data. The other cards are not affected. Report the message text to your administrator.

2.4 Colours

The colours follow one rule throughout:

Colour	Meaning
Grey	Zero, or nothing to do
Green	All clear, or good news (paid claims, nothing outstanding)
Amber	Something is waiting for someone
Red	Something has gone wrong and needs action

2.5 Clicking

There are two kinds of click:

- **Click anywhere on a card** to open the grid it is about — for example the Encounters grid from the *Needs attention* card.

- **Click an underlined line** to open the list behind that one number. When the mouse is over such a line the pointer turns into a pointing finger. A line is underlined only when its count is above zero: there is nothing to open behind a zero.

A list opened from a line (for example *Claims with error tags*) belongs to the dashboard. It hides when you open a grid and comes back, where you left it, when you return to the dashboard.

3 The Billing dashboard

The Billing module shows up to four cards, followed by two charts when there is room on the screen.

3.1 Today

The number of encounters dated today, one line per site, and at the bottom how many of today's encounters are still unsigned and how many orders were placed.

On a day with no encounters yet — early morning, a weekend — the card says so and shows the last day that had activity instead, so you are never looking at a bare zero wondering whether something is broken.

Clicking the card opens the Encounters grid.

3.2 Needs attention

The big number is the count of **encounters in the last 7 days that have no patient signature**. It is amber while anything is outstanding and green at zero. The bottom line shows the date of the oldest one still waiting.

Below it, depending on your permissions, two more lines:

- **Possible duplicate patients · new in 90 days.** Groups of patient records that look like the same person (same last name, first name, birth date and city) where at least one record was created in the last 90 days. Click the line to see the list. Nothing is merged or changed from that list; it tells you who to look at.
- **Claims with error tags · last 7 days.** See the next chapter.

Clicking the card itself opens the Encounters grid.

3.3 Billing

This card covers the **last 90 days** of active encounters:

- **Unbilled encounters** (amber) — encounters with no claim. Click the line to open the Unbilled Encounters grid on those same 90 days. See the chapter *Unbilled encounters*.
- **Claims rejected in the last 90 days** (red) — claims whose most recent status is a rejection, counted by the date the rejection was recorded, not by the encounter's date. A rejection that arrives this week on a claim from an older encounter is work to do this week, so it is counted. Click the line to open the list.
- **Paid claims** (green).
- **Claims with no status · served in the last 90 days** — primary claims with no response recorded yet, counted by the claim's own date of service (any encounter, active or not). Secondary claims are not counted here. Click the line to open the Claims grid with its filter already set to exactly these claims: *Missing Flag*, *Primary claims only* and *Srv From / Srv Thru* on the same 90 days.
- **of N encounters · last 90 days** — the total the unbilled and paid figures are counted from. The rejected and no-status figures have their own basis, above.
- **Upcoming encounters pre-created** — encounters already entered for future dates. They are shown for information and are **not** counted as unbilled.

The bottom line gives the oldest unbilled encounter, the oldest rejection still unworked (by the date it was rejected) and the oldest claim with no status. Those dates are the ones to watch: an old date here is money at risk of passing the payer's filing deadline.

3.3.1 The rejected claims list

The list shows each rejected claim with the patient's record number, the patient, service date, insurance, charges, the date it was rejected, where the rejection came from and **the reason**. When the rejection came

from the clearinghouse or the payer, the reason is their own wording. When the claim was denied on a remittance (an 835), the reason is built from the adjustment and remark codes on that remittance.

Double-click a row to open the claim, correct it, and resubmit; double-click the *MedRec* column instead to open the patient's record, when the fix is there (an insurance card, a date of birth). **Refresh** re-reads the list after you have worked a few; **Print** gives you a paper copy to work from.

3.4 System health

One line per background service your site uses. A green dot means all is well, amber means "look at this", red means it has stopped or failed.

- **HL7 interface** — whether the interface is listening, when the last message arrived, and how many messages are waiting to be processed. Red if the listener has not been heard from, or if no message has arrived since the last day the clinic was busy.
- **Modality worklist** — how many entries are scheduled, whether the worklist server is up, and when a scanner last asked for its list.
- **AutoTasks** — when the background service last ran a cycle and when the last backup was taken. Red if AutoTasks has stopped reporting or the last backup **FAILED**; amber if the last good backup is more than two days old. AutoTasks also keeps the CMS reference data current and emails what it fetched — see **The reference data keeps itself current** in the claim checks chapter.
- **Received today** — remittance files, claim statuses and eligibility responses that arrived today.

Any red line here should be reported to whoever looks after your server the same day. A failed backup in particular must not wait.

3.5 The charts

When the window is wide enough, two bar charts follow the cards:

- **Billed vs collected · last 12 months** — what was billed and what was collected, month by month.
- **Encounters per month · last 12 months.**

On a smaller screen the charts may not fit; maximise the main window to see them.

4 Claims with error tags

4.1 What the line counts

When a claim is saved, Mp10 runs a set of checks on it — diagnosis codes and modifiers that do not agree, a payer the patient does not have on file, a date of birth the insurer reports differently, codes this payer has refused before, and others. A check that finds a problem puts a **tag** on the claim: a short coloured label with the name of the problem, such as *Wrong Side ICD10* or *Insurance DOB Mismatch*.

The **Claims with error tags · last 7 days** line on the *Needs attention* card counts the primary claims that carry at least one of these tags and were saved in the last seven days. Secondary claims are not counted or listed: Mp10 creates them from the primary after its payment, and the problem a tag names is fixed on the primary. Claims already fully paid (green flag) or overpaid are left out as well: a tag on them can no longer prevent a rejection. Tags that a user added by hand are not counted. Every check, and the tag each one places, is described in the next chapter, *The claim checks*.

The count, and the list behind it, cover every tag a check places: the diagnosis, modifier, NCCI, payer and denial-history tags alike. Two tags are left out on purpose: *Unattached Patient Payments*, which is not an error in the claim, and any tag a user added by hand.

Each of those claims is likely to be rejected or denied if it goes out as it is. Working this line before the claims are sent is far cheaper than working the rejections afterwards.

4.2 Opening the list

Click the line. A list opens with one row per claim, newest first:

Column	Shows
Claim	The claim number, with the claim's status flag in front of it — the same flags the Claims grid shows (green paid, red rejected, and so on). No flag means the claim has not been sent
MedRec	The patient's record number. **Double-click it to open the patient**
Patient	Last name, first name
Service date	The claim's date of service
Insurance	The payer
Tags	Every tag on the claim, separated by semicolons
Saved by	The user who last saved the claim
Saved	When it was last saved

The list title says how many claims it holds.

4.3 Opening a claim and finding out why

Double-click the claim number (or any column other than *MedRec*) to open that claim, just as it opens from the Claims grid.

Double-click the MedRec column to open the patient's record instead, just as it opens from the Claims grid. Several tags point at the patient rather than the claim — *Insurance Sex Mismatch*, *Insurance DOB Mismatch*, *Payer Not On Patient* — and the fix is made on the patient's demographics or insurance tab. Opening the patient from here means the biller does not have to leave the list, look the patient up in the Patients grid, and come back. A patient already open in the Claims grid is brought to the front rather than opened twice.

The claim's tags are shown along the tag strip of the claim window. Each tag is only a name; to see what the check actually found:

- **Hover** the mouse over a tag. A tooltip shows the first finding in one short line — for example which line's modifier disagrees with which diagnosis. When there is more to say, the tooltip ends with **"(click for more)"**.
- **Click** the tag. A window lists **every** finding for that tag, in full.

The explanation is worked out from the claim **as it is now**, not as it was when the tag was added. So:

- If you have already corrected the problem, the explanation says the check finds nothing on the claim any more, and **the tag clears the next time the claim is saved**.
- If the explanation still describes a problem, the claim still has it.

4.4 Clearing a tag

1. Read the explanation (hover or click the tag).
2. Correct the claim — the diagnosis, modifier, payer, patient's insurance record, or whatever the explanation points to. When the problem is on the patient's record, double-click the *MedRec* column to open it from the list.
3. **Save** the claim. The checks run again; a tag whose problem is gone is removed.

Some tags call for judgement rather than a correction. *Denied Before*, for instance, says this payer has often refused these codes; the claim may be perfectly right and simply need supporting documentation. In that case the tag can stay: it is a warning, and the claim can still be sent.

Press **Refresh** on the list after working a few claims to see what is left. The count on the dashboard updates on its next refresh.

5 The claim checks

This chapter lists every check Mp10 runs on a claim, the tag each one places, what the tag means and what to do about it.

5.1 How a tag is placed

- **The checks run every time a claim is saved** in the claim window. The diagnosis-side checks also run as you edit the procedure lines and diagnoses, so their tags can appear before you save.
- **Each check starts by removing its own tags** and adds a tag back only if it still finds the problem. The tags on a saved claim therefore always describe the claim as it was last saved. Fix the problem, save, and the tag is gone.
- **A tag never stops a claim from being saved or sent.** Each is a warning. The coder or biller decides, because the claim may be correct and the rule may not apply to it.
- **A check places a tag once per claim**, however many lines have the problem. Hover over the tag or click it to see every line it is about.
- **Three checks also offer a correction** when you save: the date-of-birth, sex and payer-on-file checks. Each asks first and changes nothing unless you answer **Yes**.
- **Tags you add by hand are different.** No check removes them. They stay until you remove them, and clicking one says it was added by a user.
- **Some checks need reference data**: the ICD-10 code set, the CMS fee schedule, the NCCI tables, and the denial history. The first three keep themselves current – AutoTasks downloads each new release from CMS and imports it (see **The reference data keeps itself current** below). The denial history is built from your own remittances: each posted remittance adds to it, and the administration module's **Update Data Dictionary** rebuilds it when a newer version of Mp10 learns something the history on file does not have yet, and skips it otherwise. **File > Rebuild Denial History** always rebuilds, and takes a few minutes. Where data is missing, the check does nothing and places no tag. A missing tag is not proof that a claim is clean.

Secondary claims that Mp10 creates automatically after a primary payment are checked when someone opens and saves them.

5.2 Summary

Tag	The claim has…
Missing ICD10s	No diagnosis codes
ICD10 Relation	A line pointing at a diagnosis that is not on the claim
Wrong Side ICD10	RT with a left-side diagnosis, or LT with a right-side one
ICD10 Side Unspecified	RT or LT with a diagnosis that leaves the side unspecified
Bilateral CPT, One-Side ICD10	A surgical code with modifier 50 and diagnoses for one side only
ICD10 Excludes1 Conflict	Two diagnoses ICD-10-CM says are never coded together
ICD10 Sequence	Diagnoses in an order a "code first" instruction contradicts
Modifier 50 Not Allowed	Modifier 50 on a code the fee schedule does not allow it on
Modifier 50, Units Over 1	Modifier 50 on a line with more than one unit
Modifier 50 With RT/LT	Modifier 50 and RT or LT on the same line
RT/LT On Bilateral CPT	RT or LT on a code that already means both sides

Tag	The claim has…
NCCI Pair, Never Together	Two codes NCCI never pays together
NCCI Pair Needs Modifier	Two codes NCCI bundles unless a modifier shows they were separate
Units Over MUE	More units of a code than the CMS limit allows
Payer Not On Patient	A payer that is not on the patient's insurance record
Insurance DOB Mismatch	A date of birth different from the one the payer reported
Insurance Sex Mismatch	A sex different from the one the payer reported, or none at all
Denied Before	Codes this payer has refused often and recently
Missing Performing Physician	No performing physician, to a payer that has refused claims sent without one
Refused For This Patient	A code this payer refused for this same patient, with nothing paid for it since

5.3 Diagnosis codes

5.3.1 Missing ICD10s

The claim has no diagnosis codes at all. Payers reject a claim without them. Add the diagnoses and save.

5.3.2 ICD10 Relation

A procedure line's diagnosis pointer names a diagnosis that is not on the claim. For example, the line points at diagnosis 3 but the claim has only two. Correct the pointer on the line, or add the missing diagnosis.

5.3.3 Wrong Side ICD10

A line carries modifier **RT** (right) and its diagnosis is for the **left** side, or **LT** with a right-side diagnosis. The modifier and the diagnosis contradict each other, and payers reject the line. Correct whichever of the two is wrong.

The check reads the **first** diagnosis the line points at. Diagnoses that have no side at all are never questioned.

5.3.4 ICD10 Side Unspecified

A line carries RT or LT, but its diagnosis is the "unspecified side" code of a family that has right and left codes. The modifier says which side; the diagnosis should too. Replace it with the side-specific code.

5.3.5 Bilateral CPT, One-Side ICD10

A **surgical** procedure (codes 10000 to 69999) carries modifier **50** (bilateral), and every diagnosis the line points at is for one side only.

The bilateral procedure is supported, and no tag is placed, when:

- the line points at a **bilateral** diagnosis, or
- it points at both a right and a left diagnosis, or
- the claim carries the **same diagnosis for the other side** (or its bilateral form) anywhere in its diagnosis list, even if the line does not point at it.

Diagnostic procedures such as imaging are not checked. On those, a one-sided finding on a bilateral exam is correct coding.

An administrator can exempt whole diagnosis families from the three side checks with the **BILLING / ICD10-LATERALITY-EXCLUDE** setting in the System Registry: a comma-separated list of code

prefixes. It is empty by default. The claim window reads it; the MpWeb overnight re-tag does not, so an exempted family can still be tagged there.

5.3.6 ICD10 Excludes1 Conflict

Two diagnoses on the claim are a pair that ICD-10-CM marks **Excludes1**: conditions that should never be reported together. The classic example is type 2 diabetes (E11.9) together with type 1 diabetes (E10.9). An Excludes1 note on a category or chapter applies to every code beneath it.

Clicking the tag names the two codes and the note that forbids them. Since 2017 the guidelines allow such a pair when the two conditions are genuinely unrelated, so this is the coder's decision. Remove the code that does not belong, or leave both if they are unrelated.

5.3.7 ICD10 Sequence

A diagnosis carries a **"code first"** instruction naming another diagnosis that is on the claim, but the claim lists them the other way round. For example, a rib fracture listed before the associated spinal cord injury, which should come first.

Clicking the tag says which code should come first. Reorder the diagnoses and save. The order on the claim is the order sent to the payer.

5.4 Modifiers and the CMS fee schedule

These checks compare a line's modifiers with the **bilateral surgery indicator** in the CMS Medicare Physician Fee Schedule. AutoTasks imports each quarter's file automatically; the MpAdmin menu remains for loading one by hand. A code the fee schedule does not list is not checked, and neither is indicator 3, on which payers disagree.

5.4.1 Modifier 50 Not Allowed

Modifier 50 is on a code whose indicator is **0**, **2** or **9**. On those codes the payer does not accept a bilateral adjustment:

- **0:** the procedure cannot be billed as bilateral.
- **2:** the code already covers both sides.
- **9:** the concept does not apply.

Remove modifier 50. If the procedure was really done on both sides of a code with indicator 0, bill each side as the payer requires.

5.4.2 Modifier 50, Units Over 1

A line with modifier 50 carries more than one unit. A bilateral procedure is one line with one unit. Set the units to 1.

5.4.3 Modifier 50 With RT/LT

Modifier 50 and RT or LT are on the same line. They are never used together: use either modifier 50 on one line, or one RT line and one LT line.

This check and the previous one need no fee schedule data, so they work at every site.

5.4.4 RT/LT On Bilateral CPT

A line carries RT or LT on a code whose indicator is **2**, meaning the code itself already describes both sides. Remove the side modifier.

5.5 NCCI edits

The National Correct Coding Initiative (NCCI) publishes which codes cannot be billed together and how many units of a code are plausible. AutoTasks imports each quarter's tables automatically; the MpAdmin menus remain for loading them by hand. Each payer is checked against either the professional or the facility table, depending on whether the payer is set up as professional.

5.5.1 NCCI Pair, Never Together

Two codes on the claim form a pair that NCCI never pays together on the same day, **whatever modifier** is used. One of them should not be on the claim. Clicking the tag names the pair.

5.5.2 NCCI Pair Needs Modifier

Two codes on the claim form a pair that NCCI bundles together, unless the second code is marked as a separate service. No tag is placed when the second code's line carries one of these modifiers:

`24  25  27  57  58  59  78  79  91  XE  XS  XP  XU`

No tag is placed either when one line is RT and the other LT.

If the two services really were separate, add the modifier that documents why. If they were not, remove the bundled code.

Both pair checks run only for payers that ask for them. An administrator turns them on per payer by ticking **Check NCCI PTP pairs** on the X12 tab of the insurance company's record. Payers without the tick are never checked for pairs.

5.5.3 Units Over MUE

A code carries more units than its Medically Unlikely Edit (MUE), the CMS limit for one patient on one day. Depending on the code, the limit applies to each line or to the claim's total for that code. This check runs for every payer.

Bilateral billing is allowed for: one RT line and one LT line of the code, or 2 units of a code whose fee-schedule indicator says it may be billed bilaterally. Correct the units, or check whether the service was entered twice.

5.6 Payer and patient

5.6.1 Payer Not On Patient

The claim's insurance, or its secondary insurance, is a payer that does not appear on the patient's insurance record. A payer counts as on file when the name matches, or when it has the same payer ID as one the patient has. Claims like this are rejected far more often than others.

When you save, Mp10 names the payers the patient does have and asks whether to **add the claim's payer to the patient's insurances**, carrying over the claim's policy number:

- Answer **Yes** if the patient's record is out of date. Then check the member number and effective date on the patient's record.
- Answer **No** if the claim is wrong. Use the magnifier beside the claim's insurance to pick one of the payers the patient has.

5.6.2 Insurance DOB Mismatch

The date of birth the claim will send differs from the date the payer reported in the patient's latest **eligibility response**. Mp10 prefers a response from the claim's own payer and says so when the response came from a different one.

When you save, Mp10 shows both dates and asks whether to use the payer's date. If you answer **Yes**, the date is written to the patient's **insurance record for that payer**, which is the date claims to that payer send. The patient's own date of birth on the patient record is **not** changed: the payer's file can be the wrong one, and the clinical record must stay as it is.

5.6.3 Insurance Sex Mismatch

The patient's sex on the patient record differs from what the payer reported in the latest eligibility response, **or the patient record has no sex at all**. A blank is a real problem: the claim then sends "unknown", and the eligibility request leaves out the patient's date of birth too.

When you save, Mp10 asks whether to use the payer's answer. If you answer **Yes**, the patient record is updated, because both claims and eligibility requests take the sex from there.

5.6.4 Denied Before

This payer has refused these codes before, **often and recently**. Mp10 learns this from remittances that paid nothing for a line, and from payer rejections. Clicking the tag names the code, the diagnosis, how often it was refused and the payer's own reason.

To avoid crying wolf, the tag is placed only when all of these are true:

Rule	Default
Refusals of the same payer, code and diagnosis	2
Refusals on at least this many different check dates	3
Refusals as a share of all this payer's remittance lines for the code	20%
The most recent refusal is no older than	6 months
Refusals counted from the last	24 months

Refusals that are not really a refusal of the service are ignored: patient responsibility, duplicates, coordination of benefits, contractual adjustments and reporting-only codes. Remark codes never count on their own.

This tag is evidence, not a rule. The claim may be entirely right this time. Check that the diagnosis supports the procedure, attach documentation if the payer usually asks for it, and send the claim.

The thresholds are System Registry settings under **BILLING**, named **DENIAL-MIN-COUNT**, **DENIAL-MIN-DATES**, **DENIAL-MIN-RATE**, **DENIAL-RECENT-MONTHS**, **DENIAL-MONTHS** and **DENIAL-IGNORE-CODES**. **DENIAL-REASONCODES** narrows the warning to a list of denial reason codes; empty, the default, counts every reason. A site that has not built its denial history never sees this tag.

5.6.5 Missing Performing Physician

The claim has no **performing physician**, and this payer has refused claims sent without one. Without it the claim goes out with no rendering provider, and whether that is accepted is up to the payer: many pay such claims, some refuse every one. So a blank field alone places no tag. Mp10 learns which payers refuse from the remittances you post: a claim with no performing physician that paid nothing with reason **206** (National Provider Identifier missing) or remark **N290** (missing rendering provider identifier).

The tag is placed when the payer has refused at least **DENIAL-MIN-COUNT** such claims (2 by default) in the last **DENIAL-MONTHS** (24). Clicking it names the payer, how many claims it refused, the last one, and the reason. Fill in the performing physician and save; the tag clears.

Auto-created claims never have a performing physician, so they carry this tag whenever their payer is one of these.

BILLING / DENIAL-PERFPHYS-CODES lists the reasons that count (**206,N290**); empty switches the check off. A claim refused for this and later corrected by filling in the physician no longer counts, so the payers are learned from the claims still left blank. A site that has not built its denial history never sees this tag.

5.6.6 Refused For This Patient

This payer has already refused one of the claim's codes **for this same patient**, and has not paid that code for the patient since. Clicking the tag names the code, the date of the refusal, and the payer's own reason — both halves of it: the adjustment code and the remark that says what was wrong, such as missing credentialing data or a modifier the payer expects.

This is the strongest warning Mp10 gives about a payer's behaviour. Measured on real data, a code the payer refused for a patient is refused again about two times out of three; once the payer has paid it, a later refusal is rare. That is why a payment clears the tag: if the payer paid the code for this patient after refusing it, nothing is shown.

Fix what the reason names before sending: an authorization, the provider's credentialing with that payer, a modifier, or a frequency limit. If the claim is right as it is, send it — the tag is a warning, as always.

The same check runs **when an order is placed** on an encounter, and a message names the refused code when the encounter is saved. The **Mp10 Orders** guide describes it, with the duplicate-order warning that runs at the same time.

It looks back as far as **BILLING / DENIAL-MONTHS** (24 months by default) and ignores the same reasons **BILLING / DENIAL-IGNORE-CODES** lists, the settings Denied Before uses.

5.7 The reference data keeps itself current

AutoTasks, the background service, checks the CMS website once a day. When CMS publishes a new quarter of the fee schedule or the NCCI tables, AutoTasks downloads and imports it. When CMS publishes the yearly ICD-10-CM code set, effective each October 1st, AutoTasks downloads and imports that too, and writes a report of every code that changed next to the downloaded files. Nobody has to remember a quarter or a year again; the MpAdmin import menus remain for loading a file by hand.

After any check that imported, downloaded or failed something, AutoTasks sends an **email** saying exactly what it did – a new ICD-10 year arriving included. A quiet check, when CMS has published nothing new, never sends mail.

The System Registry entries behind this:

Entry	Default	Meaning
AUTOTASKS / FETCH_CMS	YES	NO switches the daily CMS check off entirely
AUTOTASKS / CMSFETCH_CHECK_HOURS	24	Hours between checks
AUTOTASKS / CMSFETCH_PATH	a cms folder beside AutoTasks.exe	Where the downloaded files and the ICD-10 change reports land
CMSFETCH / NOTIFY_EMAIL	empty	Who gets the result email; commas between addresses

With **CMSFETCH / NOTIFY_EMAIL** empty, the email goes to **ERRORREPORTS / NOTIFY_EMAIL** instead. With both empty, no email is sent and the AutoTasks log still records everything. AutoTasks must have its outgoing mail (SMTP) settings configured, as for backup notices.

5.8 Switching a check off, or narrowing it

Most checks have no off switch: they warn, they never block, and a tag costs nothing when the claim is right. Where a site can turn one off or narrow it:

Check	How
NCCI Pair, Never Together / Needs Modifier	Per payer, **off by default**: tick **Check NCCI PTP pairs** on the X12 tab of the insurance company's record
Units Over MUE, the modifier-50 checks	No switch once the tables are loaded. **AUTOTASKS / FETCH_CMS = NO** stops future quarters from loading
The three side checks	**BILLING / ICD10-LATERALITY-EXCLUDE**: diagnosis-code prefixes to leave alone (claim window only)
Denied Before	The **BILLING / DENIAL-...** thresholds above; a very high DENIAL-MIN-DATES silences it, DENIAL-REASONCODES narrows it
Refused For This Patient	No switch. **BILLING / DENIAL-MONTHS** shortens how far back it looks; **DENIAL-IGNORE-CODES** names reasons that never count
Missing Performing Physician	**BILLING / DENIAL-PERFPHYS-CODES** empty switches it off; DENIAL-MIN-COUNT and DENIAL-MONTHS set how many refusals it takes
Everything else	Always on. The DOB and sex checks only speak when an eligibility response exists to compare against

5.9 Tags you may see on older claims

Earlier versions placed **Missing Right ICD10** and **Missing Left ICD10**. Those checks were replaced by the side checks above. The old tags are removed automatically the next time such a claim is saved.

6 Unbilled encounters

6.1 What an unbilled encounter is

An **unbilled encounter** is an active encounter dated in the last 90 days that has **no claim**. Every one of them is one of two things:

- a visit that happened and has **not been billed yet** — revenue the practice has earned and not asked for; or
- an encounter that should **not** be billed at all, most often because the patient **did not show up** after the encounter had been created.

Encounters created ahead of time for future dates are not counted; the card lists them separately as *upcoming encounters pre-created*.

6.2 Why it matters

- **Filing deadlines.** Every payer gives a limited time after the date of service to submit a claim. An encounter left unbilled past that limit is revenue that can no longer be collected. The **"Oldest unbilled since"** date at the bottom of the Billing card tells you how close the oldest one is.
- **A clean count is a useful count.** If no-shows are left in the list, the number grows with encounters nobody will ever bill, and the real unbilled visits hide among them. A billing team that trusts the number to mean "visits still to bill" can work it to zero every week.
- **Reports and statistics.** Encounters that never took place inflate visit counts, productivity figures and census reports until they are marked inactive.

Aim to keep the number low and the oldest date recent. Look at it daily; work it at least weekly.

6.3 Working the list

Click **unbilled encounters** on the Billing card. The Unbilled Encounters grid opens on the same 90 days the card counted. Its **Ordered** column says what each patient came for, from the encounter's orders; the orders icon beside the date opens them, to check or to add orders while billing. An encounter with nothing in Ordered had no orders entered, and has to be billed from the paperwork. The **Mp10 Orders** guide explains why entering orders at registration speeds billing up. For each encounter:

- **If the visit took place**, bill it as usual. Once it has a claim it leaves the list.
- **If the patient did not show up**, or the encounter was created by mistake, mark it **inactive** as described below.

6.4 Inactivating a no-show encounter

1. In the Unbilled Encounters grid (or the Encounters grid in the Patients module), **double-click the encounter** to open it.
2. On the first tab of the encounter window, tick **Inactive encounter**. In the institutional Patients module the box is worded *Inactivate the encounter*; it is the same setting.
3. **Save** the encounter.

Inactivating is the only way to take a visit out of the lists: encounters are never deleted, so their history, signature and any payment stay on file.

What happens to an inactive encounter:

- It **leaves the unbilled count** and the dashboard's other encounter counts on the next refresh.
- It is **kept**, not deleted. Its history, notes and documents remain, and it still appears in the patient's record — shown in **grey** in encounter lists.
- It is left out of reports that count active encounters.

- **No charges can be posted** to it. If you later find the patient was in fact seen, open the encounter, clear **Inactive encounter**, save, and bill it as usual.

To see inactive encounters in the Unbilled Encounters grid, tick **Show Inactive Encounters** in the grid's side panel.

Inactivate only encounters that genuinely will not be billed. An encounter that is merely late in being billed belongs in the list until it is billed; marking it inactive would hide revenue the practice is owed.

7 The Patients dashboard

Patients10 shows a dashboard for admissions and the record room. All counts cover **active encounters in the last 90 days** unless the card says otherwise.

7.1 Encounters by Source

The number of encounters in the last 90 days, one line per source, largest first; smaller sources are grouped as *other*. Clicking the card opens the Encounters grid.

7.2 Encounters missing something

Of the encounters in the last 90 days, how many have:

- **no ICD-10 coded**,
- **no patient signature**,
- **no insurance**, and
- **no eligibility** — insured encounters for which no eligibility response is on file. This line appears only at sites that run eligibility checks.

Each is a gap that will hold up the claim later. The card reads "Nothing outstanding." when all four are zero. Clicking it opens the Encounters grid.

7.3 Patient records

Of the patients seen in the last 90 days, how many have:

- **no HIPAA signature** on file, and
- **incomplete demographics** — no birth date, a missing address, city or ZIP code, or no usable telephone number at all.

The third line, **possible duplicate patients**, counts groups of records that look like the same person across **all** patients on file. Click it to open the list. Clicking elsewhere on the card opens the Patients grid.

7.4 Incomplete records

The number of open incomplete records, broken down by how long ago the encounter took place: 0–7 days, 8–30 days, 31–90 days, and over 90 days (in red). This card has no 90-day limit — an old incomplete matters more than a new one.

7.5 Incompletes by physician

The five physicians with the most open incomplete records.

Clicking either incompletes card opens the Incompletes grid, for users who have access to it.

8 Quick reference

You see	Do this
Claims with error tags above zero	Click the line, double-click each claim, hover or click each tag, correct, save
Unbilled encounters above zero	Click the line; bill the visits that happened, inactivate the no-shows
Old “Oldest unbilled since” date	Work that encounter first: its filing deadline is closest
Rejected claims above zero	Click the line, read the reason, double-click to correct and resubmit
Claims with no status above zero	Click the line; check the oldest first, and resubmit any claim the payer never received
Possible duplicate patients	Click the line and review the records with whoever maintains patient records
Unsigned encounters	Collect the signature before the claim goes out
Red line in System health	Tell your server administrator the same day
“The refresh has stopped”	Close and reopen the module; report it if it recurs
A card you expected is missing	Ask your administrator to check your permissions